



PROCEDURE

Recognising and Responding to Acute Deterioration (RRAD) Physiological and Mental State

Scope (Staff):	NMHS Inpatient Mental Health Staff
Scope (Area):	NMHS Inpatient Mental Health Services

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1. Aim

The aim of this procedure is to provide clinicians with direction to meet the minimum mandatory requirements for recognising and responding to acute physiological and mental state deterioration for all patients receiving care within North Metropolitan Health Service (NMHS) Mental Health (MH) inpatient services in accordance with the [WA Health Mandatory Policy Recognising and Responding to Acute Deterioration Policy \(MP 0171/22\)](#) and the [NMHS Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#).

This Procedure describes the responses required by clinical staff to ensure that acute physiological and mental state deterioration is recognised and responded to appropriately and should be read in conjunction with the following documents:

- The Australian Commission on Safety and Quality in Health Care (ACSQHC) [Clinical Care Standards](#) (CCS) for Acute Anaphylaxis, Sepsis, Delirium, Stroke and Cognitive Impairment.
- ACSQHC National Safety and Quality Health Service (NSQHS) Standards [Standard 8: Recognising and Responding to Acute Deterioration](#), 2nd Ed.
- NSQHS Standards Advisory AS 19/01: Recognising and Responding to Acute Deterioration Standards; [Recognising deterioration in a person's mental state](#)
- The [National Standards for Mental Health Services](#) (NSMHS), and
- The [Sustainable Health Review \(2019\)](#) Enduring Strategy 2 recommendation that mental health care must be integrated with physical health care to improve health and wellbeing outcomes for patients.

2. Risk

Non-compliance with this procedure may result in a breach in mandatory policies, failure to meet the expected standards, cause harm to patients, their families and carers, and/or damage to the reputation and image of NMHS MH as identified in the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

3. Definitions

Acute deterioration	Physiological or mental state changes that may indicate a decline in the health status of the patient.
Delirium	Delirium is a serious condition, often unrecognised that develops over a short period of time (usually hours to a few days) and is associated with increased mortality. It is characterised by disturbances in consciousness, attention, cognition and perception.
Deterioration in mental state	defined as a “ <i>Change in a person’s perception, cognitive function or mood which negatively influences their capacity to function as they would typically choose</i> ”. Change can be gradual or acute; it can be observed by health professionals, reported by the patient themselves, or reported by their family or carers (ACSQHC Mental Health Scoping Study 2014).
Mental state	A patient’s mental state broadly refers to a person’s intellectual capacity, emotional state, and general mental health, based on clinical observation, interviewing and assessment. Mental state comprises mood, behaviour, orientation, judgement, memory, problem solving ability and contact with reality.
MH inpatient sites/services	Refers to Sir Charles Gairdner Mental health Services (MHS) Older Adult MHS (Osborne Lodge & Selby Lodge), Forensic MH (Frankland Unit & Dyandra) and Graylands Hospital.
Observation & Response Chart (ORC)	ORC refers to is a multi-tiered individualised observation and response chart used in MH inpatient services. Six physiological parameters are identified to record vital signs with colour coding to indicating mandatory escalation that guides medical and nursing actions (Refer to PMR38).
Person-centred care	Person-centred care is recognised as a foundation for safe, high-quality individualised health care that seeks to understand what is important to individuals, families, carers and support people by respecting and responding to the preferences, needs and values of consumers (ACSQHC).

Trauma Informed Care (TIC)	TIC is an approach that acknowledges the impact of trauma on individuals and promotes safety, healing and recovery. For more information see the Blue Knot Foundation's Practice Guidelines portal (link is external) .
Therapeutic engagement	Clinical interactions that are trauma-informed, sensitive to the diverse needs of consumers and are recovery focused and empowers individuals to actively participate in their own care through collaboration and shared decision making (MHCC, 2023).

4. Mental Health Clinical Governance

The [NMHS Recognising and Responding to Acute Deterioration Policy](#) mandates that all NMHS sites and services must have a committee within their clinical governance structure responsible for the monitoring and evaluation of early recognition and response to acute physiological and mental state deterioration processes for their service in accordance with the requirements of the [WA Health Mandatory Policy Recognising and Responding to Acute Deterioration Policy \(MP 0086/18\)](#) and the NSQHS Standards Standard 8: Recognising and Responding to Acute Deterioration.

The MH Clinical Governance Committee is responsible for ensuring processes are in place to monitor and ensure compliance with NMHS RRAD Policy. This includes the endorsement of clinical pathways, tools and processes to assist staff recognise, respond and escalate acute physiological and mental state deterioration, including the transfer of care when required, in accordance with the requirements of the MH RRAD Procedure.

5. Principles of Care

The [Charter of Mental Health Care Principles](#) in the [Mental Health Act 2014](#) (MHA 2014) that are required to ensure that the rights of consumers, families and carers are met and acknowledges the importance of consumer lived experience and their preferences, aspirations, values and skills as endorsed by the WA Chief Psychiatrist (chiefpsychiatrist.wa.gov.au).

All MH inpatient services utilise a MH endorsed Observation and Response Chart (PMR38)) that allows clinicians to monitor and document vital signs and any changes that occur over time including, at time of initial assessment, when a patient is experiencing or at risk of experiencing an episode of acute deterioration and prior to inter or intra hospital transfers.

The ACSQHC [standard 8](#): RRAD identifying that acute deterioration can be physiological, mental, or both, requiring health services to have systems in place to recognise and respond to patients' physical and mental deterioration in a timely manner. Clinicians are advised to use clinical judgement and consider an individual's circumstances, in consultation with the patient, family, carer or guardian.

The Clinical Handover identifies patients who are at risk of physical and mental deterioration and includes the communication of information relevant to the management of their care. Clinicians must follow the iSoBAR process during interactions with other staff and when undertaking transfer of care in accordance with the requirements of the [WA DoH Clinical](#)

[Handover Policy \(MP 0095/18\)](#), the [WA DoH Clinical Handover Matrix](#), the [NMHS Clinical Documentation Policy](#) and the [MH Clinical Record Documentation Procedure](#).

The NM RRAD Policy adheres to the guiding principles described in the National Consensus Statement: Essential elements for recognising and responding to clinical deterioration and the National Consensus Statement: [Essential elements for recognising and responding to deterioration in a person's mental state](#).

Standard Infection Prevention and Control measures along with transmission-based precautions are adhered to as required ([NMHS Infection Prevention and Control Policy](#))

6. Acute Deterioration in Physical Health

6.1 Monitoring for acute deterioration in physical health

All patients of NMHS MHS will have their physical health assessed and monitored in accordance with the [WA State-wide Standardised Clinical Documentation](#) requirements to ensure current and ongoing health needs are identified and managed in a timely and effective manner and to promote optimal physical wellbeing.

This policy establishes the minimum requirements for the physical healthcare of patient/consumers within MH services and promotes collaboration and engagement with consumers, families, support persons and carers as well as other healthcare providers. Refer to the [MH Physical Healthcare Management Policy](#).

All MH inpatient services must use the endorsed Observation and Response Chart (ORC PMR38) to document and monitor vital signs during the following events:

- during initial assessment (admission) and at times specified on the ORC
- when a patient is recognised at risk or experiencing an episode of acute deterioration
- when there is a change in medication which may impact on a patient's physical/mental state
- when the patient is given sedation or appears sedated (to monitor respiration rate hourly or more frequently if specified)
- prior to inter-hospital or intra-hospital transfer

All patients admitted to an inpatient unit will have a physical examination at the time of admission or within 12 hours of admission. The initial component of the Physical Examination (SMHMR903) can be completed by an appropriated qualified and experienced nurse with the remainder of the form to be completed by medical staff (Refer to [SSCD](#) requirements).

Medical staff must document the plan for monitoring observations including the frequency of observations, taking into consideration the patient's diagnosis, the presence of comorbidities, prescribed medications, treatments, investigations and additional requirements. The ORC includes a prompt for clinicians to consider Sepsis as a possible contributing factor. Medical Staff initiate or alter the frequency of the clinical observations on the ORC and must also record this information in the patient's medical record.

The nurse records vital signs on the ORC in accordance with the 'General Instructions' and escalates for senior nurse and medical review when readings are outside the normal parameters (Refer to ORC PMR38).

The frequency of observation should be consistent with the clinical situation of the patient. The National Consensus Statement: Essential elements for recognising and responding to Acute Physiological Deterioration recommends that the following vital signs should be monitored at least every 6 hours and recorded on the ORC (PMR38):

- Respiratory rate
- Oxygen saturation
- Heart rate
- Blood pressure
- Temperature
- Level of Consciousness
- Signs of sepsis

Individual or specific groups of patients may require monitoring of additional health parameters e.g. new onset of agitation, confusion or behaviour changes.

6.1.1 Standardised observation, screening, assessment and monitoring tools

Examples of specific screening, assessment and monitoring tools used in MHS to inform physical health care for mental health consumers include the following:

- Clinical Alert Form (PMR001)
- Alcohol and Other Drugs (AOD) withdrawal Charts
- Blood sugars (e.g. Diabetes)
- Blood tests (pathogens/microorganisms e.g. UTIs)
- Bowel Chart
- Delirium Pathway (ACSQHC)
- Dysphagia Assessment, Screening and Monitoring Tool (PMR 14A & 14B)
- Falls Risk Assessment and Management (FRAMP)
- Fluid Balance Chart
- Glasgow Coma Scale (GCS)
- Pain Scale (Abbey Pain Monitoring Scale)
- Skin integrity/wound care (Braden Scale)
- Sleep Chart

Refer to relevant physical health [policies](#) for further information on clinical tools used in MHS.

Patients undergoing treatment for physical health deterioration who have not previously been identified as being at high risk of deterioration in their mental state may also experience mental health deterioration and clinicians must remain vigilant and alert for changes in mental state.

6.1.2 Modifications

The Observation and Response Chart (ORC) must be appropriate and modifiable for individual patient circumstances. Altered parameters on the ORC must be made by the Treating or Duty



Psychiatrist or Medical delegate and are based on the patient's clinical status and or treatment and must be documented on the ORC and in the integrated progress notes.

The Clinical Management Plan for altered parameters includes:

- a monitoring plan
- an escalation plan that identifies actions required for the altered response criteria
- a time frame for which the modification is valid for up to 72 hours.

If on review the clinical rationale and management plan remains unchanged, the altered parameters may be re-endorsed for a further 72 hours by the Treating or Duty Psychiatrist or Medical delegate.

6.1.3 Advance Care Planning and/or Goals of Patient Care (GoPC)

Health care professionals must refer to any advanced care planning documentation available when managing or escalating care for a deteriorating patient. Refer to [WA Health Advanced Care Planning website](#).

The GoPC establishes the most medically appropriate and agreed goals of care that will apply in the event of physical health deterioration, during an episode of care in an inpatient mental health service as recorded on the Goals of Patient Care eForm (MR00H.1).

For specific information about the completion and application of GoPc refer to the [NMHS Goals of Patient Care Policy](#) and the [MH Goals of Patient Care Procedure](#).

6.2 Escalation of acute physiological deterioration in physical health

An escalation process is identified on the ORC and is initiated when a patient's physical health deteriorates. The MH ORC (PMR 38) identifies the criteria and/or actions required in response to acute deterioration for inpatients in mental health facilities. Clinicians record changes in the physiological parameters for patients within the ORC and respond using the escalation triggers within the ORC. The ORC is kept up to date with all changes entered correctly and contemporaneously by nursing and medical staff involved in the care of the patient. When a coloured zone on the ORC is registered, the corresponding colour coded 'Actions required' must be initiated and these actions must be documented in the patient's medical record.

6.2.1 The escalation process

Authorises the clinician to escalate care until they are satisfied an appropriate response has been made.

If the nominated clinical staff member is unable to respond to a clinical review request, the calling clinician **MUST** escalate care to the next escalation level.

Highlights the inclusion of the 'worried' criterion which allows all clinicians to escalate care in the absence of other abnormal observations; and takes into consideration the concerns of the patient, family and carers.

The escalation process includes identification of the following:

- the abnormal physiological parameters (coloured zones)

- the action required for a particular abnormal physiological parameter

Escalation Process for a Deteriorating Patient:

- Increased surveillance
- Senior Nurse review
- Urgent Medical review (Treating or Duty Psychiatrist or medical delegate)
- Medical Emergency Team response
- Code Blue Activation

Patients who repeatedly trigger during increased surveillance or Senior Nurse review despite interventions should have the response criteria escalated to the next level. Where vital signs fall within two or more different colour areas for the same time period, the actions required for the darker colour apply. Refer to the ORC for detailed instructions relating to escalation and response (PMR 38).

In the event of a patient's continuing deterioration, clinicians must consider if the deterioration could be due to sepsis, pneumonia, pulmonary embolus, cardiac arrhythmia, electrolyte imbalance, acute coronary syndrome, acute stroke or overdose / oversedation (Refer to MH Sepsis Pathway; [MH Dysphagia Management for Inpatient Policy](#) and [MH Dysphagia Screening for Inpatient Procedure](#)).

6.3 Rapid response system for acute deterioration in physical health

Mental health services have response criteria and formal rapid response systems in place to facilitate appropriate and timely response to acute physiological deterioration. Refer to ORC 38 for response criteria and actions required relating to the coloured zones under the following heading:

- Increased surveillance
- Senior Registered Nurse review of the patient
- Urgent Medical review of the patient
- Criteria for activating a Medical Emergency Team (MET)
- Code Blue Activation criteria
- Code Blue Response - Dial 55 and request an urgent medical ambulance.

Assess, respond and monitor using **ABCDE** process

- **Airway** - If required, head tilt, jaw thrust to open airway; suction; insert nasopharyngeal airway
- **Breathing** - Monitor respiratory rate and oxygen saturation.
- **Circulation** - Monitor pulse, blood pressure, and ensure IV access. Use an ECG monitor if patient is tachycardic, bradycardic or pulse is difficult to palpate.
- **Disability** – including drugs, diabetes and documentation Access LoC using APVU as per ORC Chart (PMR38)
- **Exposure** – taking in to account the dignity and personal preferences of the patient (refer to NMHS Chaperone Policy)

The MET and Code Blue rapid response consists of appropriately trained doctors and nurses in the delivery of Basic Life Support (BLS) and Advanced Life Support (ALS) working within their

scope of practice with competencies to recognise and respond to physiological and mental state deterioration and are available to respond 24/7.

Use the **DRSABC** Australian Resuscitation guide when providing an initial response. Refer to the [MH Code Blue Medical Emergency Procedure for Inpatients](#), the [NMHS Mandatory Training Policy](#) and the [MHPHDS Mandatory Training Policy](#).

6.4 Other conditions that contribute to acute deterioration in physical health

6.4.1 Acute Anaphylaxis

- Anaphylaxis is a potentially life-threatening condition and if not recognised early and treated promptly may lead to death.
- Early recognition and identification of Anaphylaxis is critical.
- Don't Delay Seek an Urgent Medical Response
- Check for any known allergies /potential risks on the Clinical Alert Form (PMR001).

Refer to the [MH Management of Clinical Alerts Policy](#) to ensure that known allergies or potential risks are documented in the clinical alerts in accordance with the [WA Health Clinical Alert MedAlert Policy \(MP 0053/17\)](#) and managed in accordance with the best evidence principles outlined in the Commission's Acute Anaphylaxis Clinical Care Standard.

6.4.1.1 Criteria for Anaphylaxis

Anaphylaxis is likely when all three of the following criteria are met:

- Sudden onset and rapid progression of symptoms
- Life-threatening Airway and/or Breathing and/or Circulation problems
- Skin and/or mucosal changes (flushing, urticaria, angioedema).

Anaphylaxis can be triggered by any of a broad range of allergens with food, drugs, stinging insects, snake venom and latex identified as possible triggers.

Exposure to a known allergen for the patient supports the diagnosis.

Food is the most common trigger in children and drugs the commonest in adults.

Virtually any food or drug can be implicated, but certain foods (nuts) and drugs (muscle relaxants, antibiotics, non-steroidal anti-inflammatory drugs and aspirin) cause most reactions. Refer to Australian Food Standard Code and HACCP Food Safety Guidelines.

6.4.1.2 Management of Anaphylaxis

The Acute Anaphylaxis Clinical Care Standard (ACSQHC) identifies six (6) quality statements for health services to follow:

- Prompt recognition of Anaphylaxis
- Immediate injection of IMI Adrenaline
- Correct Positioning
- Access to personal adrenaline injector in all healthcare settings
- Observation time following Anaphylaxis

- Discharge Management and documentation
- Refer to the DRSABC Australian Resuscitation guide when providing an initial response and assessment of the patient
- Patients with a suspected anaphylaxis should be monitored for signs of airway, breathing and circulation difficulties with access to a MET/Code Blue response
- Use the MH Anaphylaxis Algorithm/Pathway (**Appendix 1**).
- Call 55 and request an Emergency Ambulance and transfer to a General Hospital
- Document in the patient medical record and update the Clinical Alert Form
- Refer to the MH Code Blue Procedure for Inpatient MHS

6.4.2 Sepsis recognition and response

Sepsis / septic shock is life-threatening and time dependant organ dysfunction caused by a dysregulated host response to infection that that can lead to tissue damage, multi organ failure and death if not recognised early and treated promptly to reduce mortality ([ACSQHC Clinical Care Standard for Sepsis](#))

6.4.2.1 Signs of Sepsis

- Sepsis is a Medical Emergency and requires time-critical treatment.
- If sepsis is suspected seek **urgent medical attention**.
- Sepsis is characterised by a set of signs and symptoms that are recognisable and indicative of acute clinical deterioration.
- If a patient exhibits signs of sepsis escalate care and initiate the Adult MH Sepsis Algorithm/Pathway and record clinical observations (**Appendix 2**)
- Call 55 and request an Emergency Ambulance and transfer to a General Hospital

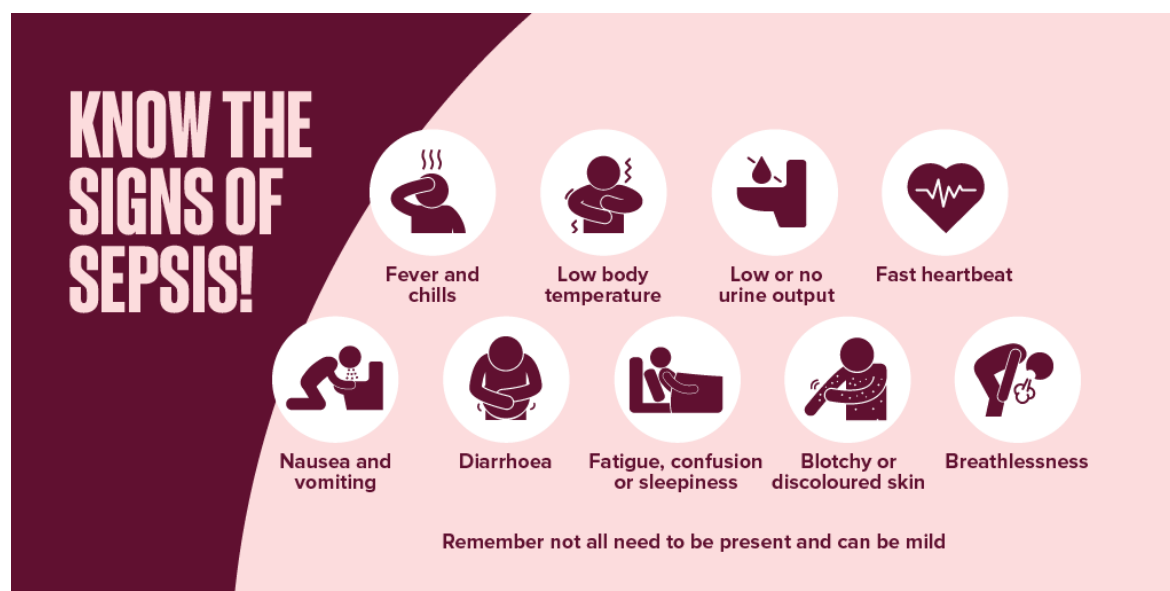


Image source: Australian Commission on Safety and Quality in Health Care

6.4.2.2 Management of Sepsis

The MH ORC include a reminder for clinicians to consider the possibility of sepsis as a contributory factor to the deteriorating patient, prompting a review of the trigger zones and the need for urgent senior nurse/medical review and /or escalation to a MET /Code blue response.

The Adult MH Sepsis Pathway is endorsed for use in mental health inpatient services in accordance with the best evidence principles outlined in the Commission's Sepsis Clinical Care Standard.

For further information refer to [NMHS Sepsis Hub](#) and the [ACSQHC Sepsis](#) CCS for additional information.

6.4.3 Stroke response

Acute stroke is a serious medical emergency, and timely treatment is critical In providing the patient the best chance of survival and recovery. With the right treatment at the right time, many people are able to recover from stroke. The FAST acronym (Face, Arms, Speech, Time) is a test to quickly identify if someone is having a stroke. Link to F.A.S.T. website.

Other symptoms of a stroke that should always be taken seriously include:

- Sudden weakness or numbness on one side of the body, including legs, hands or feet.
- Difficulty finding words or speaking in clear sentences.
- Sudden blurred vision or loss of sight in one or both eyes.
- Sudden memory loss or confusion, and dizziness or a sudden fall.
- A sudden, severe headache.

Identifying stroke patients triggers a priority dispatch transfer by Ambulance directly to a tertiary hospital for urgent assessment and management in accordance with the best evidence principles outlined in the [ACSQHC Acute Stroke Clinical Care Standard](#) and the [MH Code Blue Emergency Procedure for Inpatient MHS](#).

6.4.4 Seizure observation and management

Assess, respond and monitor using ABCDE process:

- Airway If required, head tilt, jaw thrust to open airway; suction; insert nasopharyngeal airway
- Breathing Monitor respiratory rate and oxygen saturation.
- Circulation Monitor pulse, blood pressure, and ensure IV access. Use an ECG monitor if patient is tachycardic, bradycardic or pulse is difficult to palpate.
- Disability – drugs, diabetes and documentation
- Complete neurological observations and access LoC using APVU as per ORC Chart
- Exposure - consider the dignity and personal preferences of the patient (refer to [NMHS Chaperone Policy](#))

Seizure observations include assessment of type and duration of seizure, level of consciousness (LoC) during seizure, motor activity and experience of sensory symptoms. Refer to the MH Seizure Management Guidelines (**Appendix 3**).

- Closely monitor the patient during the post seizure period until they return to baseline.
- Clinical observations continue as directed by the MO and the nurse records clinical observations and seizure activity until the patient is medically stable. Refer to ORC PMR38.
- Record seizure activity on the MH Epileptic Seizure Monitoring Chart PMR46.



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- Document the seizure event in the integrated progress notes. Refer to the [NMHS Clinical Documentation Policy](#) and the [MH Clinical Record Documentation Procedure](#).

6.4.5 Acute Mental Health deterioration

Patients with acute physical health deterioration may also experience mental deterioration and in addition to recognising and managing their physiological deterioration, prompt identification and management of mental health deterioration is also required in accordance with best practice and available evidence.

Deterioration in mental state may relate to several predisposing or precipitating factors, including mental illness, psychological or existential stress, physiological changes, cognitive impairment (including delirium), intoxication, withdrawal from substances, and emotional response to social context and environment. Change can be gradual or acute; it can be observed by health professionals, reported by the patient themselves, or reported by their family or carers.

Mental deterioration is characterised by a negative change in a patient's mood or thinking, marked by a change in behaviour, cognitive function, perception or emotional state and may impact on the person's emotional wellbeing and decision-making capacity. For additional information refer to Acute Deterioration in Mental Health Section 7.

6.4.6 Delirium

The goal of the [ACSQHC Delirium - Clinical Care Standard](#) is to improve prevention of delirium in patients at risk and to improve the early diagnosis and treatment of patients with delirium to reduce the severity and duration of the delirium. Patients identified as having delirium following screening and assessment will be monitored closely and have their care escalated (Refer to [MH Role of the Allocated Nurse](#) and the [MH Clinical Observation Procedure](#)).

Delirium is communicated at clinical handovers, is recorded in iSOFT and is documented in the patients' medical record and management plan in accordance with the requirements of the DoH Clinical Handover Policy and SSCD. Patients at greater risk of developing delirium include patients aged 65 years and older; patients with severe medical illness; patients with a hip fracture and patients with cognitive impairment (Refer to section 7 for more information on Delirium).

7. Acute deterioration in Mental Health

Deterioration in Mental Health is defined as "Change in a person's perception, cognitive function or mood which negatively influences their capacity to function as they would typically choose." (ACSQHC).

7.1 Monitoring for acute deterioration in mental state

All mental health inpatient services use endorsed tools to assess and monitor deterioration in mental state in accordance with the requirements of the MHA 2014 and WA Health SSCD framework. Mental health assessment consists of a comprehensive mental health assessment that includes a mental state examination to establish the 'baseline' behaviour and level of functioning for the patient. Refer to the [MH Clinical Risk Assessment and Management Policy](#).

Identification of early warning signs assist clinicians in recognising the onset of acute deterioration in mental state and the opportunity to collaborate with patients in the Consumer



Safety Plan and level of nursing observations required in accordance with the [MH Clinical Observation Procedure](#).

Information is obtained from the patient and with the permission of the patient collateral information is sought from family, carer, nominated person/s and other key people involved with the patient. Baseline Mental Health parameters are recorded on the Mental Health Assessment Form, the Risk Assessment and Management and Consumer Safety Plan (SSCD).

Baseline physiological observations are recorded on the Observation and Response Chart (ORC) PMR38 and on the Physical Health Assessment Form (SSCD). This information is used to assist the team in recognising acute mental state deterioration including changes in physical health status. Changes are observed and reported and when clinical deterioration is recognised care is escalated for senior nurse review and medical review as required.

Refer to SSCD Mini-Mental State Examination and the Mini Mental State Examination (MMSE) and the [Chief Psychiatrist's Clinical Care Standards for mental and physical health monitoring](#).

The Treatment Support and Discharge Plan (SMHMR 907) and the Consumer Safety Plan (SMHMR 932) must be revised and updated at transitional points in care as they represent times of potential increased risk due to transfer of care and changes to the treating team, environment, location and management plan (SSCD).

Specific assessment tools and consideration of issues impacting on the health status of vulnerable / at risk individuals include Child & Adolescent MHS suite of documents, consideration of LGBTI+ and recognition of risks of suicide, self-harm, mental disorders and other risk factors, such as abuse, homelessness and substance. Refer to the National LGBTI Health Alliance (2020) and SSCD Mental Health Framework (2021).

SSCD FDV screening tools are used to identify if the patient is experiencing family and domestic violence (FDV950) requiring further assessment and follow up in situations where the patient has disclosed violence and abuse (FDV951).

7.1.1 Standardised observation, screening, assessment and monitoring tools

Standardised assessment and monitoring tools are used to provide evidence of change and assist with diagnosis, treatment, risk management, care planning and treatment support and discharge planning.

The Management Plan is updated to reflect changes in mental state and identification of interventions and treatment. When changes are recognised, it is important to check in and listen to any concerns that the patient, family, and carer may wish to discuss to ensure that potential triggers are not missed and provide information to the patient, family and carers regarding changes in the management plan.

The multidisciplinary team utilises SSCD and other validated clinical tools in the delivery of evidence-based care that is individualised to best meet the needs of the patient. Examples of specific tools used in MHS to guide patient assessment and management include the following:

- MH Agitation and Arousal PRN Scale (PMR 60D)
- MH Clinical Observation Form (PMR29)
- MH Observation and Response Chart (ORC PMR38)

- Mental State Deterioration – RAISE (Recognise, Alert, Intervene, strategies, Evaluate)
- AOD assessment and monitoring tools (ASSIST; DAST) and AOD Withdrawal Charts
- Cognitive 7 Dementia Scale (RUDAS)
- Glasgow Coma Scale
- Montreal (MoCA) Cognitive Assessment Test (mild cognitive decline & dementia)
- Dynamic Appraisal of Situational Aggression (DASA) Tool
- Risk Assessment and Management Plan (RAMP)
- Treatment Support and Discharge Plan (TSDP)
- Consumer Safety Plan (CSP)
- SSCD Physical Health Assessment (including waist circumference for Metabolic Syndrome)

7.2 Escalation of acute deterioration in mental state

Recognising change in mental state relies on having baseline information to which a person's current mental state can be compared as evidenced by Mental State Assessments that are required to be completed whenever there is a change in a person's mental state, a change in their legal status or a change in the phase of care (higher or lower observation area).

Information is routinely recorded progressively during the period of hospitalisation and on discharge using mandatory State-wide Clinical Documentation. Validated assessment, screening and monitoring tools are endorsed for use in the service. Clinical information is documented in the integrated progress notes in the patient medical record and recorded in accordance with SSCD requirements.

A review of care, treatment and management plan is required when deterioration in a person's mental state is recognised. The review includes completion of a formal risk assessment identifying potential stressors and events contributing to the change (RAMP).

Deterioration in mental state can occur at any time without obvious warning hence the need to ensure that clinical observation and monitoring is undertaken at the frequency identified so that subtle changes are observed and reported for senior nurse review and escalation for medical review as required.

Clinical deterioration may be associated with known triggers, cues or behavioural changes identified during assessment, observations or conversations with patient, family / carer or from information contained in referral documentation, previous admission or identified in the Consumer Safety Plan.

The allocation of a nurse to observe and monitor patients at risk facilitates early recognition of deterioration, implementation of de-escalation strategies and timely communication with the MDT (PMR29).

Nursing Clinical Observations facilitate rapport building, developing trust and establishing a therapeutic relationship ([MH Clinical Observation Procedure](#)).

Recognising clinical deterioration in the mental health status of a patient requires active exploration and review of a number of factors contribution to mental health deterioration as indicated below:

- Findings as indicated on the Agitation and Arousal Chart (PMR60)
- Escalate to senior nurse for clinical review
- Risk Assessment and management Plan((SMHMR905)
- Identification of de-escalation strategies
- Review of routine and PRN medications
- Use of PRN Medication
- Review of Clinical Observations
- Identifying triggers and implementing strategies that are documented in the Consumer Safety Plan and were previously effective during escalation/arousal (SMHMR932).

7.3 Rapid response system for acute deterioration in mental state

Response to acute deterioration in mental health may include the following:

- Increased surveillance
- Senior Nurse review
- Medical review by Medical Officer/Registrar/Consultant (MER)
- Code Black Activation
- Code Blue Activation (in the event of medical emergency)
- Consideration of patient transfer to specialist / tertiary services
- Contacting next of kin, nominated person, family/carers
- Completion of clinical documentation and mandatory notifications
- Refer to the MH Agitation and Arousal Chart (PMR 60D) and RAISE – Mental State Deterioration (**Appendix 4**).

Utilising information obtained from the patient, family and carers, from direct observation, from undertaking formal assessments and accessing patient data will assist clinicians to complete comprehensive assessment, diagnosis, risk assessment, management planning and implementation of care and treatment that is appropriate for the patient.

Specific interventions may focus on the following:

- Increase clinical observations and use of de-escalation techniques in response to emotional arousal, increased agitation and verbal or physical aggression.
- Review the frequency and or level of nursing observations and request a Senior Nurse assessment.
- Discuss the use of routine and PRN medications with senior nurse and request a review by medical staff.
- Implement supportive strategies to promote self-regulation of emotions, behaviours, physiological and psychological symptoms using individually tailored therapeutic interventions (Mindfulness, Relaxation, Sensory Modulation).
- Review level of risk and escalate for urgent medical review if concerned.
- Consider relocating the patient to a lower stimulus area to provide individualised support if safe to do so (Refer to Risk Assessment and Management Plan (RAMP) and Agitation and Arousal Chart (PMR 60).
- Monitor and document the outcomes of interventions used.
- The effectiveness of the interventions used must be continuously assessed, reviewed and modified depending on the individual response of the patient and ongoing assessment of risk.

-
- Document actions in ORC, SSCD, Incident Reports, Progress Notes and other assessment and management tools used

A Code Black response may be activated in response to acute deterioration such as escalation of threatening or harmful behaviours to self, others or damage to the environment (Refer to [MH Clinical Risk Assessment and Management Policy](#); code Black Escalation Criteria in MH Emergency Management Manual and the [MH Management of Restrictive Practices \(formerly Seclusion & Restraint\) Policy and Procedure](#)).

A Code Blue response may be activated in response to acute self-harming and life-threatening behaviours (refer to Code Blue Activation Criteria in MH Emergency Management Manual (red folder) and the [MH Code Blue Procedure](#)).

7.4 Other conditions that contribute to acute deterioration in mental health

Clinical deterioration may be related to other factors that can contribute to both mental and/or physical deterioration and will trigger an escalation process in accordance with the ORC parameters and Mental Health Assessment tools that assist clinicians in recognising and responding to mental state deterioration.

- Medically unexplained symptoms or abnormal illness or behaviour.
- Patients with Eating Disorders who are medically compromised e.g., Body Mass Index (BMI) <14 along with other abnormal health parameters. Refer to WAEDOCs Clinicians Guide for Assessing Medical ([WAEDOCs](#)).
- Mental health patients with underlying medical conditions and co-morbidities. Refer to the Assessment and Physical examination modules and assessment tools for Metabolic Monitoring identified in SSCD Framework (SMHMR903 & CAMHS003).
- The Clinical Guidelines for the Physical Care of Mental Health Consumers (2010) describes metabolic syndrome as a recognised cluster of features predictive of both cardiovascular disease and Type 2 diabetes including obesity, glucose intolerance/insulin resistance, hypertension and lipidemia. Metabolic Syndrome is associated with several factors including, but not limited to medication, lifestyle factors, psychosocial factors and comorbid physical illness.

7.4.1 Cognitive Impairment

Cognitive impairment (e.g., dementia, delirium and neurological disorders cognitive) may be identified and confirmed with the use of specialist investigations and tests and assessment tools.

Examples of SSCD tools to assess cognitive functioning used in MHS:

- Montreal Cognitive Assessment Scale (MoCA)
- Rowland Universal Dementia Assessment Scale (RUDAS)

Alcohol and other substance disorders and withdrawal syndrome as identified using validated tools such as the Alcohol Use Disorders Identification Test (AUDIT) and Drug Abuse Screening Test (DAST-10) Refer to SSCD.

7.4.2 Delirium

Delirium is a serious and potentially life-threatening condition. An acute change in mental status that develops over a short period of time, usually hours or days.

Delirium is characterised by the rapid onset of impaired attention that fluctuates, together with impaired cognition and/or altered consciousness, perceptual disturbances and changes in behaviour. Whilst delirium is more common in people over the age of 65, it is important to recognise that people under 65 may also be affected.

The [ACSQHC Delirium Clinical Care Standard](#) provides highlights the need for early identification of patients at risk to improve the early diagnosis and treatment of patients with delirium and reduce the severity and duration of delirium:

- early identification of risk
- interventions to prevent delirium
- patient-centred information and support
- assessing and diagnosing delirium
- identifying and treating underlying causes. Refer to the CCS Delirium: Fact Sheet (**Appendix 5**).

Patients with dementia may experience deterioration in their mental state (such as agitation, aggression, psychosis) and these may be attributed to behavioural and psychological symptoms of their dementia. It is therefore advised that a comprehensive assessment is undertaken to exclude delirium, pain and other physical problems ([ACSQHC St.8, RRAD](#)).

The ACSQHC Pathway for patients with Cognitive Impairment (delirium and dementia) is used to provide clinical guidance for clinicians in inpatient MH services as follows:

- identify patients at risk of developing delirium using the ACSQHC Delirium/Dementia Pathway (**Appendix 6**)
- high-risk groups for delirium include patients aged 65 years and 45 years and over for Aboriginal and Torres Strait Islanders
- patients with a severe medical illness, hip fracture and UTI. Refer to Sepsis Algorithm/Pathway (**Appendix 2**)
- assessing patients for cognitive impairment using recommended SSCD assessment tools; 3MS Mini Mental State (MMS) SMHMR914 and the Rowland Universal Dementia Assessment Scale (RUDAS) SMHMR915 (SSCD)
- investigate and treat the causes of mental state deterioration (Screening tools; MSE, MMSE)
- use strategies that may assist in the care of at-risk patients, for example, preventing and treating pain, reducing sleep disturbance, review of medications used, treating dehydration, and infection sources;
- providing a supportive environment that is aimed at addressing the needs of the patient and family holistically;
- use patient centred approaches to manage transitions of care effectively (individualised interventions and management plans)
- provide training and education of staff; and resources such as appropriate equipment should be readily accessible.

Refer to the [ACSQHC Clinical Care Standards Delirium](#): Delirium Pathway (**Appendix 6**) for more information on evidence-based guidelines on the assessment and management of delirium.

7.4.2.1 Health Outcomes for Older Adults experiencing delirium

- Length of stay doubles.
- Rates of falls, incontinence and pressure areas triples.
- Three times more likely to need institutional care within six months.
- Likelihood for Dementia within three years triples.
- Higher mortality rates of 25-33%.

Clinicians must communicate, support and provide information to patients, families, carers and identified others relating to Delirium (Refer to Delirium Fact Sheet **Appendix 5**). It is important that clinicians ask if families, carers and others have noticed any changes or have concerns. (Refer to [NMHS Aishwarya's CARE Call Policy](#)) and [NMHS Good Practice Guidelines for Engaging with Families and Carers in MHS](#)).

Additional information and processes for managing delirium is available from the ACSQHC Clinical Care Standards for [Delirium](#) and Cognitive Impairment.

8. Communication with family, carers, nominated persons and other identified persons

Relevant family members, careers and other identified persons are informed of changes in care including any deterioration, treatments provided and /or transfer of care of the person in a timely manner and contact (or attempts to contact family) is documented in the patient medical record. This will include:

- advising family and carers of changes in care, treatment, location and any physical or mental deterioration in the patient's condition in a timely manner;
- notifying family and carers if the patient is transferred to another area of the hospital or to another hospital / facility;
- providing family and carers with a response if they raised their concern about the patient's health or treatment provided;
- when transferring care all pertinent information is included in written and verbal information and is communicated to the patient / family / carers and to referral services.

Risks are identified and clearly communicated in both verbal and written documentation with particular attention to impacting information for older adults, Aboriginal people, people with cognitive deficits and other vulnerabilities.

Refer to [MH Communication with General Practitioners \(GP\) and Shared Care Policy](#)

9. Aishwarya's CARE Call

Aishwarya's CARE Call is a 24/7 service that patients, family and carers can access when they have concerns or are worried about the care that is provided in DoH hospitals.

Aishwarya's CARE Call provides patients, their families and carers with a pathway to request assistance when they feel that the healthcare team has not fully recognised the patient's changing health condition.

Patients, their families or carers are often aware before clinical staff of subtle changes in their or their loved one's health, but may not feel comfortable, or know how to share their concerns with clinical staff.

Aishwarya's CARE Call supports established strategies to improve the recognition and response to clinical deterioration, including the use of NMHS ORC with defined escalation requirements and site-specific clinical escalation processes.

All patients / family / carers are kept informed about the patient's physical and mental health status in accordance with the consent of the patient and the Aishwarya's CARE Call process. Refer to [WAEDOCS Clinicians Guide for Assessing Medical Risk](#).

Refer to [NMHS Aishwarya's CARE Call Policy](#) for information and resources that are made available to patients, families and carers in mental health inpatient facilities.

10. Staff Education and Training

NMHS MH provides education on recognising and responding to acute deterioration to all clinical and non-clinical staff, within the scope of their role, on the commencement of their employment, and via regular ongoing education sessions.

In addition, all staff should be made aware of relevant local policies, rapid response system and required documentation, and know how to activate the local rapid response system in accordance with the requirements of the [NMHS Employee Induction and On-boarding Policy](#) and [NMHS Mandatory Training Policy](#).

Staff are required to comply with the [MHPHDS Mandatory Training Policy](#).

All clinical and non-clinical staff must receive education about:

- their local escalation protocol as relevant to their position
- how to call for emergency assistance if they have any concerns about a patient.
- MHS has processes in place to ensure that education programs are regularly evaluated

For more information refer to the NMHS Mandatory Training Policy and the MHPHDS Mandatory Training Policy.

11. Compliance, monitoring and evaluation

The MH Clinical Governance Committee has responsible to ensure that processes are in place for the monitoring and evaluation of early recognition and response to acute physiological and mental state deterioration (Refer to [NMHS RRAD Policy](#)).

The monitoring and evaluation of early recognition and response to acute physiological and mental state deterioration is undertaken as part of the NMHS MH Annual Documentation Audit and reported to the MH Executive through the MH Safety Quality and Performance Framework



Regular review and evaluation of the effectiveness of local rapid response systems are undertaken as part of the SQ Audit Calendar and reported to Leadership Teams for review and action. Reports are tabled at the Mental Health Clinical Executive Team.

- Monitoring and ensuring local compliance with this Policy and associated site specific procedures / clinical practice guidelines.
- Review activity and data reports (Datix CIMS, MET/Code Blue, ORC and Audit reports).
- Monitor compliance with MHPHDS Mandatory Training including Code Blue and Code Black Training.
- Implement and monitor MH Goals of Patient Care.
- Implement and monitor Aishwarya's CARE Call.

For more information refer to the NMHS ORC Audit Tool, MHPHDS Audit Tools MHPHDS [SQPU Hub page - Audits](#).

Related internal policies, procedures and guidelines

DOH

[WA Health Mandatory Policy Recognising and Responding to Acute Deterioration Policy \(MP 0171/22\)](#)

[WA DoH Clinical Handover Policy \(MP 0095/18\)](#)

[WA DoH Clinical Handover Matrix](#)

[WA State-wide Standardised Clinical Documentation](#)

NMHS

[NMHS Recognising and Responding to Acute Deterioration \(RRAD\) Policy](#)

[NMHS Clinical Documentation Policy](#)

[NMHS Goals of Patient Care Policy](#)

[NMHS Chaperone Policy](#)

[NMHS Aishwarya's CARE Call Policy](#)

[NMHS Mandatory Training Policy](#)

[NMHS Good Practice Guidelines for Engaging with Families and Carers in MHS](#)

[NMHS Infection Prevention and Control Policy](#)

[NMHS Sepsis Hub](#)

MH

[MH Clinical Record Documentation Procedure](#)

[MH Physical Healthcare Management Policy](#)

[MH Goals of Patient Care Procedure](#)

[MH Dysphagia Management for Inpatient Policy](#)

[MH Dysphagia Screening for Inpatient Procedure](#)

[MH Code Blue Medical Emergency Procedure for Inpatients](#)

[MH Management of Restrictive Practices \(formerly Seclusion & Restraint\) Policy and Procedure.](#)

[MH Communication with General Practitioners \(GP\) and Shared Care Policy](#)

[MH Clinical Observation Procedure](#)

[MH Role of the Allocated Nurse](#)
[MH Clinical Risk Assessment and Management Policy](#)

References

[Mental Health Act 2014](#)
[ACSQHC Clinical Care Standards Delirium](#)
[ACSQHC St.8, RRAD](#)
[ACSQHC Delirium - Clinical Care Standard](#)
[ACSQHC Acute Stroke Clinical Care Standard](#)
[ACSQHC Clinical Care Standard for Sepsis](#)
[National Standards for Mental Health Services](#)
[Charter of Mental Health Care Principles](#)

Useful resources

[WA Health Advanced Care Planning website](#)
[chiefpsychiatrist.wa.gov.au](#)
[Chief Psychiatrist's Clinical Care Standards for mental and physical health monitoring](#)
[NMHS Sepsis Hub](#)
[Essential elements for recognising and responding to deterioration in a person's mental state.](#)
[the Blue Knot Foundation's Practice Guidelines portal \(link is external\)](#)
[SQPU Hub page - Audits](#)
[Aishwarya's CARE Call flyer for Aboriginal people](#)
[Aishwarya's CARE Call posters](#)

Policy Sponsor	Director of Nursing MHPHDS				
Policy Contact	Director of Nursing MHPHDS				
Date First Issued:	14/02/2014	Last Reviewed:	23/07/2024	Review Date:	23/07/2027
Endorsed by:	Director of Nursing MHPHDS			Date:	23/07/2024
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety ☐  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☒  Std 8: Recognising and Responding to Acute Deterioration				
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice Consumer and Carer Involvement in Individual Care Physical Health Care of Mental Health Consumers Risk Assessment and Management Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
14/02/2014	1.0	Clinical Practice Improvement Coordinator	Initial endorsement
28/02/2018	2.0	Clinical Practice Improvement Coordinator	Content and process changes to align with DoH Clinical Deterioration Policy and DoH Clinical Handover Policy. Scheduled review.
16/05/2018	2.1	Policy Officer	Updated HealthPoint hyperlinks
27/06/2018	2.2	A/Policy and Audit Officer	Updated hyperlinks to align with updated DoH Recognising and Responding to Acute Deterioration Policy and Guideline
09/10/2018	2.3	A/Policy and Audit Officer	Inclusion of NMHS MH and higher-order Clinical Care of People who may be Suicidal policy documents.
03/01/2019	2.4	A/Policy and Audit Officer	Updated DoH policies and NSQHS Standards V2.
05/02/2019	2.5	A/Policy and Audit Officer	Minor amendment – Sponsor SQP Manager approved: Reference “Adult Observation and Response Chart (AORC) and “ORC” due to updated Chart in 2018. Removed “every 8 hours” for the management plan must be reviewed and modified in the Management Plan Section”. Removed “on the ORC and “at point 3 in the Roles and Responsibilities” section as an intervention section is not on the updated ORC, Addition of the Take 5: ORC 2018 in the useful resources section.
10/06/2022	3.0	Director of Nursing MHPHDS	Changed from Procedure to Policy, change of title from Clinical Deterioration Procedure to Recognising and Responding to Acute Deterioration (RRAD), All current information and new evidence included: NSQHSS Consensus Statements for Standard 8, Aishwarya's Care Call, Goals of Patient Care and SEPSIS Guidelines.
13/08/2024	4.0	Director of Nursing MHPHDS	Major review to align with NMHS RRAD Policy, new procedure combining mental health and physical health deterioration for mental health consumers. Update of information relating to current tools and resources for screening, assessing and monitoring clinical deterioration in MHS.

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD Number: 788).

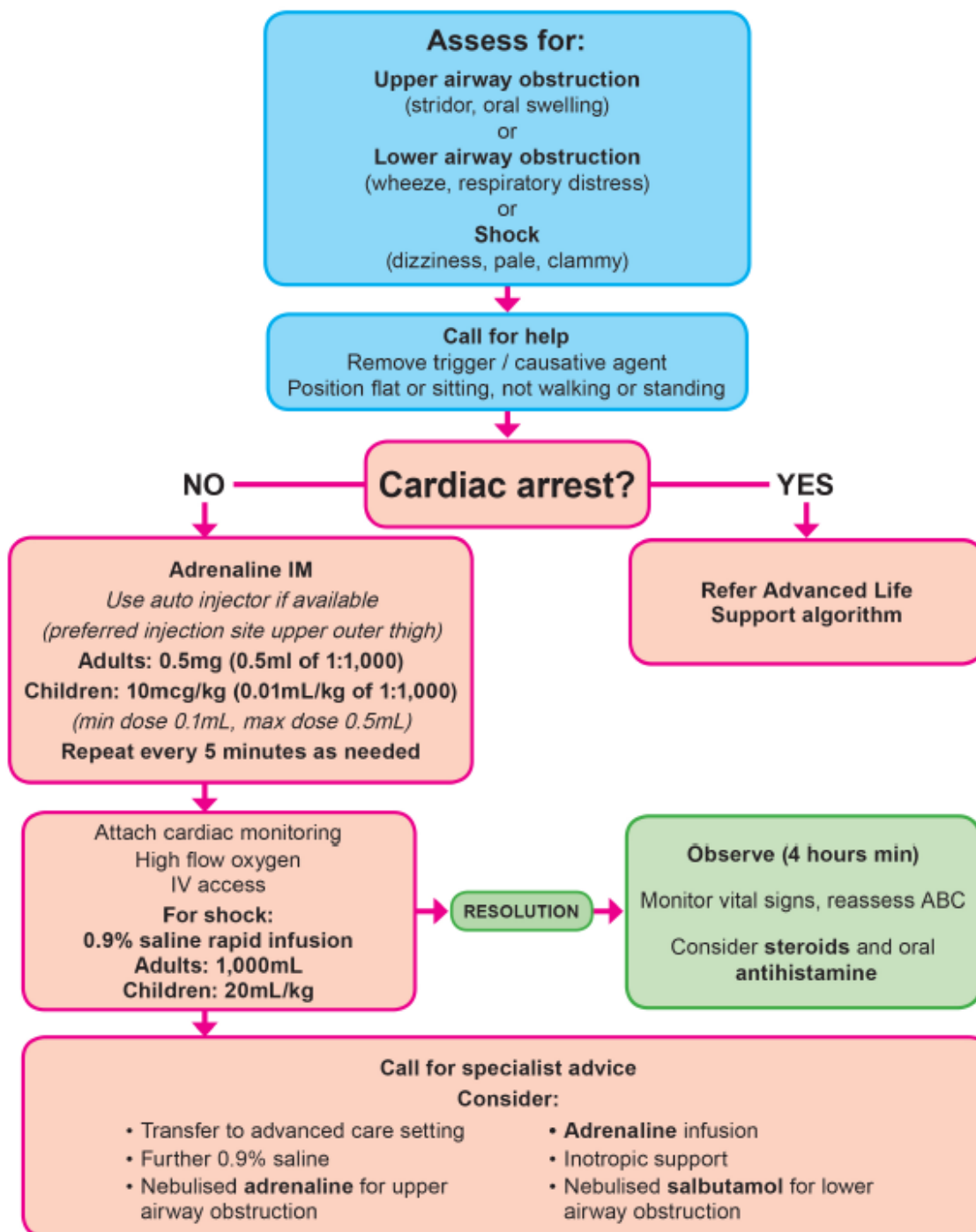
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Appendix 1: Anaphylaxis Algorithm / Pathway

Anaphylaxis



Reviewed August 2023



NEW ZEALAND
Resuscitation Council
WHAKAHAUORA AOTEAROA

Appendix 2: MH Adult Sepsis Pathway (pg 1 of 2)



XC100140

Hospital / Health Service Mental Health Adult Sepsis Pathway Ward / Dept: _____ Doctor: _____	Surname	UMRN / MRN	
	Given Name	DOB	Gender
	Address		Post Code
	Telephone		

Adult Sepsis Pathway for use in all mental health inpatient facilities

ARE YOU CONCERNED THAT YOUR PATIENT COULD HAVE SEPSIS?

Use this Pathway **ONLY** if history and examination indicate **SEPSIS** is the likely diagnosis

RECOGNISE

Consider the following sepsis risk factors

☐ Re-presentation within 48 hours	☐ Immunocompromised
☐ Recent surgery or wound	☐ Age > 65 years or > 45 years ATSI
☐ Indwelling medical device	☐ Fall

Absence or presence of risk factors does not exclude or confirm sepsis as a cause of deterioration
(Consider other causes and seek prompt assistance from senior staff if the patient is deteriorating)

Does your patient have any new onset of the following signs and symptoms of infection?

☐ Fever or rigors	☐ Line associated infection/redness/swelling/pain
☐ Dysuria/frequency	☐ Abdominal pain/distension/peritonism
☐ Cough/sputum/breathlessness	☐ Altered cognition

PLUS

RESPOND & ESCALATE

Any Adult Observation and Response Chart PURPLE ZONE observation OR response criteria?

- ☐ SBP < 90mmHg
- ☐ Lactate ≥ 4mmol/L
- ☐ Base excess < -5.0

Any Adult Observation and Response Chart RED or ORANGE or Yellow ZONE observations OR response criteria?

- ☐ Respirations ≤ 9 or ≥ 25 per minute
- ☐ SpO₂ < 95%
- ☐ SBP < 100mmHg
- ☐ Heart Rate ≤ 50 or ≥ 110 per minute
- ☐ Temperature ≤ 35°C or > 38°C
- ☐ Altered LOC or new onset of confusion

Assess LOC using Alert, Voice, Pain, Unresponsive (AVPU)

YES

Patient has SEVERE SEPSIS or SEPTIC SHOCK until proven otherwise

- Sepsis is a medical emergency
- Call for a Medical Emergency Response (MER) unless already made
- Conduct targeted history and clinical examination

YES

Patient may have SEPSIS

- Call for a Medical Review unless already made
- Conduct targeted history and clinical examination
- Obtain **SENIOR CLINICIAN** review to confirm diagnosis and prioritise investigations and management.
- Does the Senior Clinician consider the Patient has Sepsis?

YES

Look for other common causes of deterioration and treat

- New arrhythmia
- Hypovolaemia/haemorrhage
- Pulmonary embolus / DVT
- Atelectasis
- AMI
- Stroke
- Overdose/Over Sedation

- Repeat observations within 30 minutes AND increase the frequency of observations as indicated by the patient's condition
- Document decision/diagnosis and management plan in health care record
- Re-evaluate for sepsis if observations remain abnormal or deteriorate

Inform the Medical Officer as per local Clinical Escalation Process. CALL MET if indicated

Discuss management plan with the patient and their family
Adapt treatment to the patient's end of life care plan if applicable
Call an Ambulance if patient requires Urgent Transfer to ED

ADULT SEPSIS PATHWAY

MHS V1.1 Acknowledgement to NSW Health and Clinical Excellence Commission - March 2022

Page 1 of 2

MH Adult Sepsis Pathway (pg 2of 2)

_____ Hospital / Health Service Mental Health Adult Sepsis Pathway Ward / Dept: _____ Doctor: _____	Surname		UMRN / MRN	
	Given Name		DOB	Gender
	Address			Post Code
				Telephone

MANAGEMENT PLAN

Patients with presumed Sepsis are at high risk of deterioration despite initial resuscitation with intravenous antibiotics and fluids. These patients require a Management Plan which needs to be discussed with the Medical Officer (MO). This plan needs to be communicated to the Treating Team, Senior Nurse, patient and patient's family and carer/s.

Specific Management Plans are to be documented in the Patient Medical Record

INITIAL 24 HOURS

Observation and Monitoring pending confirmed diagnosis of SEPSIS

Prescribe the frequency of observations ☐

(Minimum recommendation every 30 minutes for 2 hours, then hourly for 4 hours)

Monitor and reassess for signs of deterioration which may include one or more of the following: ☐

- Respiratory rate in the Purple, Red, Orange or Yellow Zone
- Systolic blood pressure < 100mmHg
- Decreased or no improvement in level of consciousness
- Urine output less than 0.5mL/kg/hr
- No improvement in serum lactate level

If deteriorating (has any Purple, Red or Orange Zone criteria), escalate as per your local Clinical Escalation Process and inform MO

Repeat lactate 4 and 8 hours post recognition ()

4 hours Date: _____ Time: _____ Result: _____ mmol/L ☐

8 hours Date: _____ Time: _____ Result: _____ mmol/L ☐

Fluid Resuscitation

Prescribe IV fluids as appropriate based on the patient's condition ☐

(Monitor for signs of pulmonary oedema)

Reassess

Confirm diagnosis and consider other causes of deterioration ☐

Check preliminary results ☐

(If patient is neutropenic, review antibiotics and change if required)

Review treatment / management

- Discuss with MO and treat accordingly ☐
- Document plan to continue, change or / cease antibiotics ☐
- Continue monitoring for deterioration including urine output ☐
- If the patient's recovery is uncertain discuss the goals of care with patient/family ☐
- Provide end of life care if identified in Goals of Patient Care
- If condition deteriorates patient may require urgent transfer to ED

24 – 48 HOURS

Reassess

- Actively seek microbiology / investigation results and review ☐
- Confirm diagnosis, document source of sepsis in the health care record ☐
- Discuss with Medical Officer ☐
- Consider seeking advice from infectious disease / microbiology physician ☐
- Document plan to continue, change or cease antibiotics ☐
- Obtain approval for restricted antibiotics ☐
- Repeat biochemistry as indicated ☐
- Continue monitoring for deterioration including urine output ☐

Continue to monitor as per patient's condition – observations, medical review, fluids, antibiotics

Appendix 3: MH Seizure Management Guidelines

Seizure Management Guidelines

Initial Assessment	Protect patient from harm.
Airway	Ensure clear airway; gentle suction if required
Breathing	Record respiratory rate
	Auscultate chest
	Monitor oxygen saturation
Circulation	Check pulse (cardiac arrest may present as seizure)
	Record blood pressure
	Monitor ECG
Disability	Assess conscious level - AVPU
	Check BGL (hypoglycaemia can cause seizures)
Exposure	Record type and duration of seizure activity
IF AIRWAY, BREATHING OR CIRCULATION COMPROMISED - activate CODE BLUE / MET call	
PROLONGED (> 5 mins) OR REPEATED SEIZURES – activate CODE BLUE / MET call	
ABC stable	Urgent MO review
Airway	Perform airway manoeuvres as required Administer Oxygen 10 Litres/minute via a Hudson Mask
Breathing	Support breathing with bag and mask device as required
Circulation	Obtain IV access & send blood for analysis (FBC, U&Es, LFTs)
	If BGL <3.5mmol/L give 50mL IV Glucose 50% (or 1mg IM Glucagon)
Seizure > 5mins	Administer Midazolam 5mg IV (10mg IM if IV not available) 2.5mg IV Midazolam advised for patients >65yo or weight <40kg
Seizure >10mins	Repeat above dose IV Midazolam 2.5-5mg - total max 10mg (20mg if IM)
Consider	Levetiracetam 30mg/kg (max 4500mg/dose)
OR	Sodium Valproate 30mg/kg IV (max 3000mg/dose)
	Phenytoin 20mg/kg IV (max 2000mg/dose)
Seizure activity continues for 30 minutes	
Initiate second anticonvulsant immediately (refer above)	
MO to assess if patient should be intubated and referred to ED for further management	
Continuing Care post seizure activity	
Patient at baseline	Closely observe patient for minimum of 4 hours
Documentation	Document seizure activity and treatment in patient notes
Follow-up:	
Consult Neurologist and consider: Optimisation of anti-epileptic medication Investigations to exclude reversible causes	

Appendix 4: Mental State Deterioration (RAISE)

R

Recognise

- Reported change
 - A person, or someone who knows the person well, reports that her or his mental state is changing for the worse.
- Distress
 - A person, or someone involved in her or his care, shows signs of distress, which are evident through observation and conversation.
- Loss of touch with reality or consequence of behaviours
 - A person is losing touch with reality or the consequences of her or his behaviour.
- Loss of function
 - A person is losing her or his ability to think clearly, communicate, or engage in regular activities.
- Elevated risk to self, others or property
 - A person's actions indicate an increased risk to self, others, or property.

A

Alert

- Inform the treating team of mental state change/deterioration

I

Intervene

- Nursing observation, assessment and review of risk
- Medical assessment:
 - Review current management plan
 - Rule out delirium
- Risk assessment: Nursing and Medical staff
 - Aggression and violence
 - Self-harm / suicide
 - Vulnerability (sexual / financial / cognitive)
 - Discharge against medical advice
- In general hospital ED setting refer to Mental Health Consultation Liaison Team
- Consult with pharmacy for medication review

S

Strategies

- Consider Nurse Initiated environmental modifications:
 - Use of Single room
 - Low stimulus environment/away from high traffic flow area
 - Closer to nursing observation/work area
- Increase nursing observations +/- nursing special
- Inform family on Aishwarya's CARE call
- Engage patient in care planning – 'what matters to you'
- Identify personalised de-escalation tools
- Implement behaviour management plan -strategies identified in the Patient Safety Plan
- Consider need for activating a code black emergency response
- Add mental health critical information to clinical handover document

E

Evaluate

- Ongoing review of nursing observations
- Ongoing liaison with Treating Team / MH Consultation Liaison Service
- Ongoing monitoring of medication regime
- Ensure mental health issues are included in discharge summary to GP
- Ensure appropriate community mental health referrals are made

Adapted from *Recognising Signs of Deterioration in a Person's Mental State* by Dr Cadeyrm Gaskin and Dr Gavin Dagley on behalf of the ACSQHC 2018

Appendix 5: Delirium Fact Sheet

AUSTRALIAN COMMISSION
ON SAFETY AND QUALITY IN HEALTH CARE



FACT SHEET
for clinicians

Delirium

Clinical Care Standard

The goal of the *Delirium Clinical Care Standard* is to improve the prevention of delirium in patients at risk and the early diagnosis and treatment of patients with delirium, in order to reduce its incidence, severity and duration.

1 Early identification of risk

A patient with any key risk factor for delirium is identified on presentation and a validated tool is used to screen for cognitive impairment, or obtain a current score if they have known cognitive impairment. Before any planned admission, the risk of delirium is assessed and discussed with the patient, to enable an informed decision about the benefits and risks.

2 Interventions to prevent delirium

A patient at risk of delirium is offered a set of interventions to prevent delirium and is regularly monitored for changes in behaviour, cognition and physical condition. Appropriate interventions are determined before a planned admission or on admission to hospital, in discussion with the patient and their family or carer.

6 Preventing complications of care

A patient with delirium receives care to prevent functional decline, dehydration, malnutrition, falls and pressure injuries, based on their risk.

7 Avoiding use of antipsychotic medicines

Antipsychotic medicines are not recommended to treat delirium. Behavioural and psychological symptoms in a patient with delirium are managed using non-drug strategies.

3 Patient-centred information and support

A patient at risk of delirium and their family or carer are encouraged to be active participants in care. If a patient is at significant risk or has delirium, they and their family or carer are provided with information about delirium and its prevention in a way that they can understand. When delirium occurs, they receive support to cope with the experience and its effects.

4 Assessing and diagnosing delirium

A patient with cognitive impairment on presentation to hospital, or who has an acute change in behaviour or cognitive function during a hospital stay, is promptly assessed using a validated tool by a clinician trained to assess delirium. The patient and their family or carer are asked about any recent changes in the patient's behaviour or thinking.

A diagnosis of delirium is determined and documented by a clinician working within their scope of practice.

5 Identifying and treating underlying causes

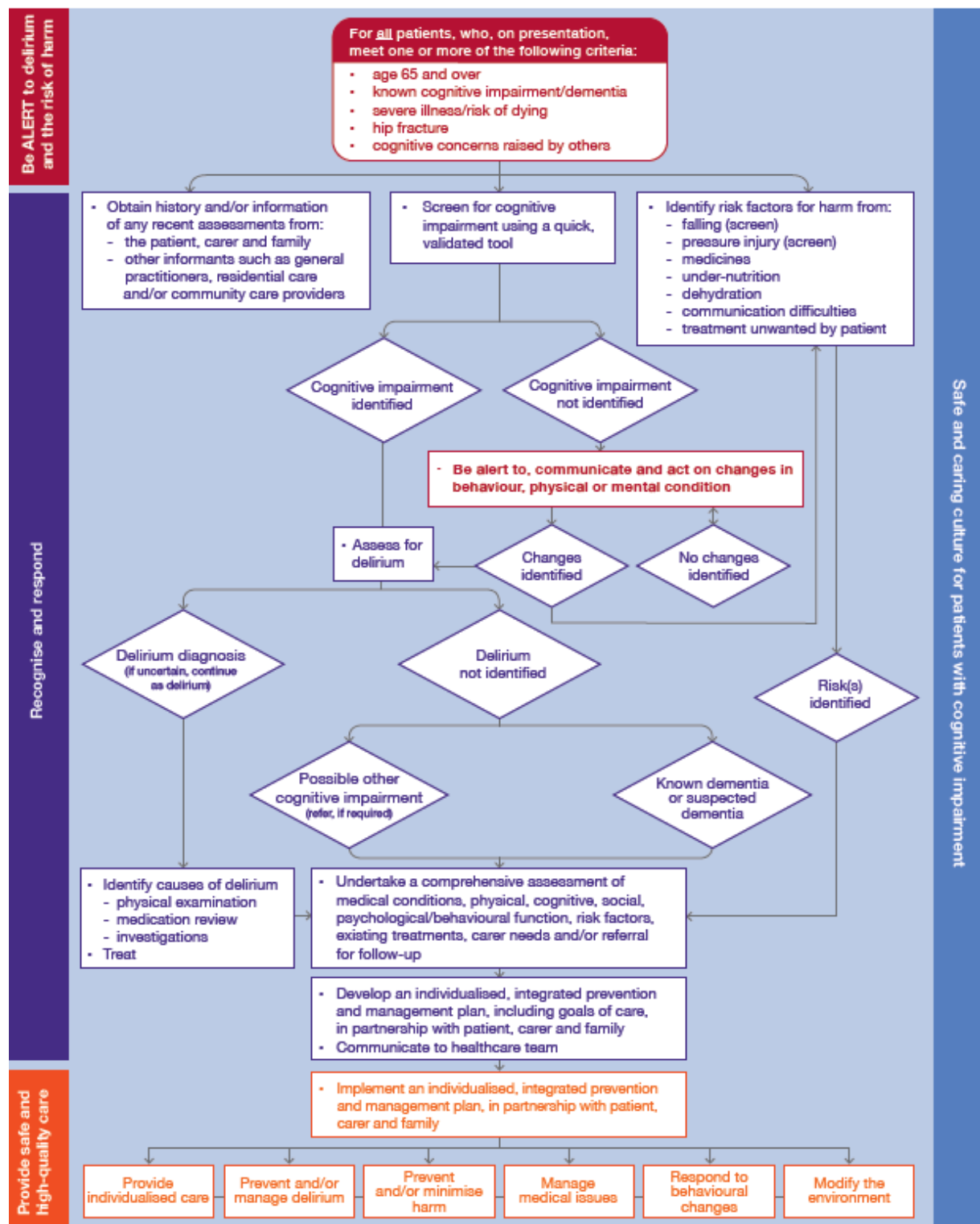
A patient with delirium is offered a set of interventions to treat the causes of delirium, based on a comprehensive assessment that includes relevant multidisciplinary consultation.

8 Transition from hospital care

Before a patient with persistent or resolved delirium leaves hospital, an individualised comprehensive care plan is developed collaboratively with the patient and their family or carer. The plan describes the patient's post-discharge care needs and includes strategies to help reduce the risk of delirium and related complications, a summary of changes in medicines and any other ongoing treatment. This plan is provided to the patient and their family or carer before discharge, and to their general practitioner and other regular clinicians within 48 hours of discharge.

Appendix 6: Delirium / Dementia Pathway (ACSQHC) (pg 1 of 2)

Safety and quality pathway for patients with cognitive impairment (delirium and dementia) in hospital



Appendix 6: Delirium / Dementia Pathway (ACSQHC) (pg 2 of 2)

Key steps in the pathway

1 Be alert to delirium and the risk of harm for patients with cognitive impairment

- Clinicians are alert to delirium and the risk of harm from cognitive impairment among patients who:
 - are aged 65 and over
 - have a known cognitive impairment or a formal diagnosis of dementia
 - have a severe illness or are at risk of dying
 - have a hip fracture.
- Clinicians are also alert when the patient, carer, family and/or other key informants raise concerns.
- A patient with cognitive impairment is supported to understand and participate in healthcare decisions. Their informed consent is obtained. If the patient is assessed as unable to provide consent, their substitute decision-maker is consulted.

2 Recognise and respond to patients with cognitive impairment

- A patient identified as being at-risk is screened for cognitive impairment. The patient's history is obtained from the patient, carer, family and/or other key informants. A patient's risk of harm from falls, pressure injuries, medicines, under-nutrition, dehydration, communication difficulties or unwanted treatment is identified.
- A patient with cognitive impairment is assessed for delirium. If delirium is present, causes are investigated and treated. If uncertain, the patient's condition is treated as delirium.
- Any change in a patient's behaviour, or physical or mental condition is acted on. If changes are observed, the patient is re-assessed for delirium and other risk factors.
- A comprehensive assessment of the patient is undertaken. If dementia is suspected and a comprehensive diagnostic process is not appropriate, the patient is referred for further assessment and follow-up.
- An individualised, integrated prevention and management plan is developed in partnership with the patient, carer and family, and communicated to the healthcare team.

3 Provide safe and high-quality care tailored to the patient's needs

- The patient's individualised, integrated prevention and management plan is implemented as follows:
 - The patient receives individualised care in partnership with the patient, carer and family.
 - The patient's medical issues are managed, including treating the underlying causes of delirium, presenting condition and any co-morbidities.
 - A patient with, or at-risk of developing, delirium has strategies implemented to prevent delirium from occurring or to limit its duration.
 - A patient with identified safety risk factors has strategies implemented to prevent and manage the risks.
 - A patient with behavioural changes is appropriately assessed and strategies are introduced to reduce distress. Antipsychotic medicine is avoided unless non-pharmacological interventions have been ineffective, the patient is severely distressed and/or the patient is at immediate risk of harm to themselves or others.
 - The hospital environment is modified to provide safe and supportive patient care.
 - The patient's healthcare information and management plan are documented and communicated to the patient, carer and all relevant healthcare providers in a timely manner and in sufficient detail, on transition from hospital to the community.